

RIVERGATE POLICE DEPARTMENT

Supplemental Investigation Report — Field Sobriety Evaluation and Arrest

Traffic Investigation Division

Original Case No.: TC-2025-1018	Reporting Officer: Officer Ramona Delacruz, RPD #2814
Supplement No.: TC-2025-1018-S01	Assignment: Patrol, Watch III (Graveyard) / Traffic Inv.
Agency: Rivergate Police Department	Date of Occurrence: 10/18/2025
ORI: CA0197400	Date of Supplement: 10/18/2025 (same tour of duty)
Bureau: Traffic Investigation	Time Report Completed: 420 hrs

1. Purpose

This supplement is filed in support of the primary Traffic Collision Report TC-2025-1018 (S. Main Street at Third Avenue, Rivergate, 10/18/2025, 0205 hrs) and documents in expanded detail the on-scene impairment investigation of Party 1 driver **Raymond T. Keeler** (DOB 04/11/1979), operator of ACTD Bus #4177 (Block 512-OWL). Matters covered: initial officer contact, observed indicia of impairment, admonishment and administration of the standardized field sobriety test battery, preliminary alcohol screening (PAS) breath result, formation of probable cause, Miranda advisement, arrest, and initiation of the evidentiary blood-draw procedure under Vehicle Code § 23612.

Nothing in this supplement is intended to alter the primary narrative in TC-2025-1018; it augments Section IX ("Investigation of driver impairment") with the details required for the DA filing package.

2. Initial Contact

I arrived on scene at 0212 hrs. After a brief exchange with Rivergate Fire Medic 11 personnel concerning the pedestrian victim (Party 2, Yolanda Reyes) and confirming that on-board passengers were being triaged by Medic 11 and Medic 14, I made contact with the operator of Bus #4177 at approximately **02:12 a.m.** at the operator's position of the vehicle. The operator identified himself as **Raymond T. Keeler**, ACTD Bus Operator (Badge 4417), and provided a California Class B commercial driver license with passenger endorsement on request. Mr. Keeler was conscious and remained belted in the operator's seat at initial contact. He was cooperative but appeared disoriented and was slow to respond to questions.

3. Observations of Impairment

While standing immediately adjacent to the operator's position and, subsequently, while assisting Mr. Keeler to step down from the vehicle to the curbside, I made the following objective observations personally:

- **Odor of an alcoholic beverage** on Mr. Keeler's breath and, more faintly, on his person. The odor was moderate and persisted throughout my initial contact and the subsequent curbside evaluation.
- **Red, watery, and bloodshot eyes**, with visibly droopy eyelids.
- **Slurred and thick-tongued speech**, with repetitive phrasing. Mr. Keeler restated the phrase "I must have dozed" three separate times during a two-minute conversation and lost track of a direct question about his shift start time before eventually answering "about ten last night."
- **Unsteady gait.** On stepping down from the operator's position at 0217 hrs, Mr. Keeler gripped the vehicle grab bar with his right hand and the door frame with his left, paused before releasing, and swayed noticeably front-to-back once on the curb. He required a corrective step to his right in order to remain upright.
- **Flushed complexion;** delayed motor response when asked to produce his license and to close the operator's cash tray.

Mr. Keeler spontaneously volunteered that he "must have dozed off" and that he "didn't see" the pedestrian or the light pole. He denied any medical condition, denied any medication other than a blood-pressure prescription taken at approximately 1900 hrs the prior evening, and denied having consumed alcohol "since dinner last night." He would not further specify quantity, timing, or location of consumption.

4. Pre-Test Admonishment and Conditions

I advised Mr. Keeler that I was going to ask him to perform a series of voluntary **field sobriety** exercises to help determine whether he was safe to operate a motor vehicle. I confirmed that he did not wear corrective lenses at the time of driving, that he had no injuries to his head, back, legs, or feet that would prevent him from performing the tests, and that he understood the instructions. He indicated that he understood and agreed to participate.

Testing was conducted at 0219 hrs on the sidewalk on the east side of S. Main Street, north of Third Avenue, on a level, dry concrete surface. Ambient temperature approximately 58°F; conditions clear and dry. The immediate area was illuminated by the headlights of Rivergate Fire Medic 14 and by my patrol unit's take-down lamps in order to compensate for the de-energized street-light standard at the NE corner (see TC-2025-1018 Section IV). Mr. Keeler was wearing soft-soled uniform work shoes (ACTD-issue). He removed nothing.

5. Standardized Field Sobriety Test Battery

The three-test National Highway Traffic Safety Administration standardized battery was administered in the sequence and manner in which I was trained (POST DUI Investigation, refresher 04/2024). Clues were counted contemporaneously and confirmed in my field notebook at conclusion of each test.

5.1 Horizontal Gaze Nystagmus (HGN) — 6 of 6 clues observed

Administered using my department-issued stimulus (pen light) held at approximately 12 to 15 inches from the subject's face, slightly above eye level. Subject instructed to follow the stimulus with his eyes only, without moving his head. Prior to test, equal pupil size and equal tracking were confirmed; no resting nystagmus observed.

- **Left eye — lack of smooth pursuit:** present.
- **Right eye — lack of smooth pursuit:** present.
- **Left eye — distinct and sustained nystagmus at maximum deviation:** present, held at maximum deviation for a minimum of four seconds.
- **Right eye — distinct and sustained nystagmus at maximum deviation:** present, held at maximum deviation for a minimum of four seconds.
- **Left eye — onset of nystagmus prior to 45 degrees:** present.
- **Right eye — onset of nystagmus prior to 45 degrees:** present.

Total HGN clues: 6 of a possible 6. No vertical gaze nystagmus was noted on the confirmatory pass.

5.2 Walk-and-Turn — 5 of 8 clues observed

Administered on a natural line in the concrete sidewalk running parallel to the curb. Subject placed in the instructional stance (right foot in front of left, heel-to-toe, arms at sides) and instructed to remain in that stance while I demonstrated and explained the exercise. Instructions given verbatim from the NHTSA-standard script; subject stated he understood.

- **Cannot keep balance during instructions:** present — Mr. Keeler broke the instructional stance twice while I gave instructions, once stepping laterally to his right.
- **Starts too soon:** present — began walking before I completed the phrase "when I tell you to start."
- **Stops while walking:** not observed.
- **Does not touch heel-to-toe:** present — visible gap (estimated one to two inches) on steps 3, 4, 6, and 7 of the first nine-step series.
- **Steps off the line:** not observed.

- **Uses arms to balance:** present — raised both arms approximately 8 to 10 inches from his sides on steps 2 and 5 of the first series and again on step 4 of the return series.
- **Improper turn:** present — executed a pivoting turn on both feet rather than the demonstrated series of small steps around the pivot foot.
- **Incorrect number of steps:** not observed on the first series (nine steps taken as instructed); on the return, ten steps were taken. I have counted this as one clue only per training convention, categorized under "incorrect number of steps."

Total Walk-and-Turn clues: 5 of a possible 8.

5.3 One-Leg Stand — 3 of 4 clues observed

Administered on the same level concrete surface. Subject instructed to stand with feet together, arms at sides, and, on my instruction, to raise one leg (foot of his choice — he elected the left) approximately six inches off the ground, foot pointed, and to count aloud in the format "one-thousand-one, one-thousand-two..." until told to stop. Test timed at 30 seconds by my wristwatch.

- **Sways while balancing:** present — noticeable side-to-side sway beginning at approximately the "one-thousand-eight" count and continuing intermittently through the test.
- **Uses arms for balance:** present — raised both arms out from the sides on two occasions (approximately six inches out).
- **Hopping:** not observed.
- **Puts foot down:** present — set the raised foot down at "one-thousand-fourteen" and again at "one-thousand-twenty-two," each time restarting on my instruction.

Total One-Leg Stand clues: 3 of a possible 4.

6. Preliminary Alcohol Screening (PAS)

At approximately **02:15 a.m.** (0215 hrs) — administered prior to the SFST battery in order to lock in a curbside breath value as close to driving time as practicable — I administered a preliminary alcohol screening breath sample on a department-issued Intoximeters Alco-Sensor FST handheld device, serial number RPD-AS-0119. Quarterly accuracy check current per the department PAS calibration log (last check 09/29/2025; next due 12/29/2025). Fifteen-minute deprivation observation was maintained from my time of contact (0212 hrs) to the sample; no burping, belching, or regurgitation observed, and Mr. Keeler had nothing in his mouth.

I advised Mr. Keeler that the PAS test is a preliminary screening tool, is voluntary, is not a substitute for the chemical test required under the implied-consent law, and that he had the right to refuse it. He agreed to provide a sample.

PAS breath result: 0.11 % BrAC (single sample; 0.11 rounded from device display).

The device produced no error codes. The result was recorded contemporaneously in my field notebook and appears on the PAS log entry for 10/18/2025. The PAS reading is a preliminary field indication and is expressly subject to laboratory confirmation via evidentiary chemical test.

7. Opinion — Under the Influence

Based on my personal observations at the scene, Mr. Keeler's own statements and admissions, his performance on the three-test standardized field sobriety battery (14 clues across three tests, including the maximum six of six on HGN), the preliminary alcohol screening reading of 0.11 % BrAC, and my training and experience as a Rivergate PD Traffic Investigations officer, I formed the opinion that Mr. Keeler was, at the time he was operating ACTD Bus #4177 on S. Main Street at 0205 hrs on 10/18/2025, under the influence of an alcoholic beverage such that he was unable to operate a motor vehicle with the care and caution of a sober person, and further that his condition was a proximate cause of the collision.

Given the injury of the pedestrian (Party 2, Reyes) and the injury of two on-board passengers (Occ-1 Cho, Occ-2 Mowbray), I identified Vehicle Code § 23153(a) — driving under the influence causing injury — as the primary applicable offense.

8. Arrest

At **02:35 a.m.** (0235 hrs) I placed Raymond T. Keeler under **arrest** at the scene for violation of Vehicle Code § 23153(a). I informed Mr. Keeler that he was under arrest, that the arrest was for driving under the influence causing injury, and that he would be transported for a chemical test and thereafter for booking. He was handcuffed (double-locked, palms out) by me and secured in the rear compartment of RPD unit 14-Adam-6.

9. Miranda Advisement

At 0236 hrs I read Mr. Keeler the Miranda advisement verbatim from my department-issued card. I asked him whether he understood each of the rights as I had explained them. He responded "yes." I asked whether, having these rights in mind, he wished to speak with me at that time. He responded, in substance, "no, not right now." I did not ask further substantive investigative questions after that point. All statements attributed to Mr. Keeler elsewhere in this supplement are either spontaneous or were made prior to the Miranda advisement, during the on-scene traffic-collision investigation described above.

10. Implied-Consent Advisement and Evidentiary Chemical Test Election

Following Miranda, I read Mr. Keeler the California Vehicle Code § 23612 implied-consent admonition verbatim from my department-issued card, advising him of his obligation to submit to a chemical test of his blood or breath, of the consequences of refusal (mandatory license suspension/revocation and evidentiary use of refusal), and of his right to elect between blood and breath (breath being unavailable for retest and blood being retained for possible retest). Mr. Keeler elected the **blood** test.

Mr. Keeler was thereafter transferred to the custody of Ofc. J. Marquez (RPD #3021), who transported him from the scene at 0244 hrs to Rivergate Regional Trauma Center for the evidentiary draw. Chain-of-custody form RPD-COC-2025-1018-01 was initiated at 0316 hrs following the draw and specimen booked at RPD evidence locker at 0402 hrs pending transfer to the Los Angeles County Sheriff's Department Scientific Services Bureau — Crime Laboratory for analysis. The analytical laboratory report is pending at the time of this filing and will be documented in a further supplement upon receipt.

11. Attachments to This Supplement

- Field notebook pages of Ofc. R. Delacruz, RPD #2814, dated 10/18/2025, entries between 0212 hrs and 0300 hrs (photocopy).
- PAS log entry, Intoximeters Alco-Sensor FST S/N RPD-AS-0119, sample time 0215 hrs, subject Keeler, R.T., result 0.11 % BrAC.
- Quarterly PAS accuracy-check certificate for device S/N RPD-AS-0119, dated 09/29/2025.
- Blood-draw chain-of-custody form RPD-COC-2025-1018-01 (initial page; balance to be filed with laboratory return).

12. Officer Certification

I certify (or declare) under penalty of perjury under the laws of the State of California that the foregoing is true and correct to the best of my knowledge and belief, based on my personal observations at the scene and my contemporaneous notes.

Reporting Officer (Print)	Ramona Delacruz
Badge / ID No.	RPD #2814

Signature	/s/ Ramona Delacruz
Date	10/18/2025
Time Signed	1418 hrs
Reviewed / Approved By	Sgt. E. Halversen (RPD #1477), Watch Commander
Reviewer Signature	/s/ E. Halversen
Date Reviewed	10/18/2025

DISTRIBUTION: Original — RPD Records (attach to TC-2025-1018); Copy — RPD Traffic Bureau; Copy — LA County District Attorney's Office (with DA filing package); Copy — ACTD Risk Management upon written request.

— END OF SUPPLEMENT TC-2025-1018-S01 —